

25NNCV08856 25NNCV08856

County: los-angeles

Division: Civil Law and Motion

Department: T

Hearing date: 2026-08-07

Source URL:

https://www.lacourt.ca.gov/tentativeRulingNet/ui/main.aspx?casetype=civil&dept_date=ALH%2CT%2C08%2F07%2F2026

Source SHA-256: cf30060ea26d11bb8974145edabef66f2ae260108fda7381b2d0e900a944e32b

Case Number: 25NNCV08856 Hearing Date: August 7, 2026 Dept: T

James R. Viglione,

individually and as successor-in-interest for Jeanne M. Viglione vs. Dignity

Health - Glendale Memorial Hospital and Health Center, et al.

Demurrer to

Plaintiff's First Amended Complaint

Moving Party: Defendant Dignity

Community Care dba Glendale Memorial Hospital and Health Center

Responding Party: Plaintiff James R. Viglione,

individually and as successor-in-interest for Jeanne M. Viglione

Tentative Ruling: Sustained

BACKGROUND

On December 12, 2025, Plaintiff James R. Viglione, individually and as successor-in-interest for Jeanne M. Viglione ("Plaintiff")

filed a complaint against Defendants Aakash N. Bodhit, M.D. ("Dr. Bodhit"),

Sameer Bajaj, M.D., Dennis G. Chang, D.O. ("Dr. Chang"), Dignity Community Care dba Glendale Memorial Hospital and Health

Center [erroneously sued as Dignity Health - Glendale Memorial Hospital and Health Center] and Doe defendants 1 through 100, alleging the following causes

of action: (1) wrongful death based on medical malpractice (CCP § 377.60); (2)

medical malpractice through a survival action (CCP § 377.20, 377.34); (3)

negligent infliction of emotional distress ("NIED"); (4) fraudulent

concealment; and (5) intentional misrepresentation/fraud.

On

April 10, 2026, the Court sustained Defendant Dignity Community Care dba Glendale Memorial Hospital and Health Center's ("Defendant" or "Dignity Community Care") demurrer to the third, fourth, and fifth causes of action with leave to amend.

On April 30, 20206, Plaintiff filed a first amended complaint ("FAC"), alleging the same causes of action in the original complaint.

Defendant now demurs to the third, fourth, and fifth causes of action.

Plaintiff opposes, and Defendant replies.

MOVING PARTY POSITION

Defendant argues that Plaintiff failed to allege sufficient facts to state a cause of action for NIED because the FAC still fails to set forth allegations that the injury-producing event was witnessed contemporaneously or that Plaintiff was aware that the decedent was being injured.

Defendant argues that Plaintiff failed to allege sufficient facts to state a cause of action for fraudulent concealment because the FAC still fails to set forth allegations establishing intent with the requisite specificity. Defendant similarly argues that Plaintiff failed to allege sufficient facts to state a cause of action for intentional misrepresentation on this basis. Defendant also argues that ostensible agency has not been sufficiently alleged.

OPPOSITION

Plaintiff contends that he sufficiently alleged facts to state a cause of action for NIED as the newly pleaded factual allegations establish that he contemporaneously witnessed a complete lack of response to the decedent's obviously deteriorating medical condition.

Plaintiff contends that the fourth and fifth causes of action for fraudulent concealment and intentional misrepresentation are sufficiently pleaded with requisite specificity based on the newly pleaded factual allegations regarding the omissions and misrepresentations made by Dr. Bodhit and Dr. Chang. Plaintiff further contends that agency allegations are sufficient at pleading stage to support respondeat superior and agency liability.

Plaintiff requests leave to amend if the Court sustains the demurrer.

REPLY

Defendant responds that Plaintiff's new allegations still show unsuccessful treatment rather than Defendant's failure to provide treatment as the underlying basis for the NIED claim.

Defendant also reiterates its arguments as to the fraud-based claims and responds that, as the named physicians are not Defendant's employees, the pleadings remain insufficient to support these claims as against it.

ANALYSIS

I. Legal Standard

A. Demurrer

A demurrer is a pleading used to test the legal sufficiency of other pleadings. It raises issues of law, not fact, regarding the form or content of the opposing party's pleading (complaint, answer or cross-complaint). (Code Civ. Proc. §§ 422.10, 589; see, also, *Donabedian v. Mercury Ins. Co.* (2004) 116 Cal.App.4th 968, 994.) It is not the function of the demurrer to challenge the truthfulness of the complaint; and for purposes of the ruling on the demurrer, all facts pleaded in the complaint are assumed to be true. (*Donabedian*, supra, 116 Cal.App.4th at 994.)

The general rule is that the plaintiff need only allege ultimate facts, not evidentiary facts. (Doe v. City of Los Angeles (2007) 42 Cal.4th 531, 550.) “All that is required of a plaintiff, as a matter of pleading, even as against a special demurrer, is that his complaint set forth the essential facts of the case with reasonable precision and with sufficient particularity to acquaint the defendant with the nature, source and extent of his cause of action.” (Rannard v. Lockheed Aircraft Corp. (1945) 26 Cal.2d 149, 156-157.)

In a demurrer, the court “may also look to exhibits attached to the complaint for operative facts. [Citation.] And because the ‘allegations that we accept as true necessarily include the contents of any exhibits attached to the complaint, . . . in the event of a conflict between the pleading and an exhibit, the facts contained in the exhibit take precedence over and supersede any inconsistent or contrary allegations in the pleading.’ [Citation.]” (Nealy v. County of Orange (2020) 54 Cal.App.5th 594, 596–97.)

Where a demurrer is sustained, leave to amend must be allowed where there is a reasonable possibility of successful amendment. (Goodman v. Kennedy (1976) 18 Cal.3d 335, 348.) The burden is on the plaintiff to show the court that a pleading can be amended successfully. (Id.; Lewis v. YouTube, LLC (2015) 244 Cal.App.4th 118, 226.) However, “[i]f there is any reasonable possibility that the plaintiff can state a good cause of action, it is error to sustain a demurrer without leave to amend.” (Youngman v. Nevada Irrigation Dist. (1969) 70 Cal.2d 240, 245.)

II. Meet and Confer

Per the declarations of Defendant's counsel D. Garrett Garberich, counsel for the parties met and conferred via telephonic conference regarding the instant demurrer and were unable to reach a resolution. (Garberich Decl. ¶¶ 8–12.)

III. Discussion

A. NIED

“[A] plaintiff may recover damages for emotional distress caused by observing the negligently inflicted injury of a third person if, but only if, said plaintiff: (1) is closely related to the injury victim; (2) is present at the scene of the injury producing event at the time it occurs and is then aware that it is causing injury to the victim; and (3) as a result suffers serious emotional distress—a reaction beyond that which would be anticipated in a disinterested witness and which is not an abnormal response to the circumstances.” (Thing v. La Chusa (1989) 48 Cal.3d 644, 667–668; accord Downey v. City of Riverside (2024) 16 Cal.5th 539, 550.) “Neither our precedent nor considerations of tort policy support requiring plaintiffs asserting bystander emotional distress claims to show contemporaneous perception of the causal link between the defendant’s conduct and the victim’s injuries.” (Downey, supra, 16 Cal.5th at p. 560.) Nevertheless, the Supreme Court’s analysis in Bird v. Saenz (2002) 28 Cal.4th 910 “remains the controlling precedent for negligent infliction of emotional distress claims arising from the provision of deficient medical care to a close relative.” (Id. at p. 560, fn. 11.)

“Bird placed particular emphasis on how difficult—often impossible—it generally is for laypersons to recognize in the moment that a course of medical treatment is causing injury. Even when plaintiffs are present at the scene of the injury-causing event and observe conduct that they later realize caused injury to a close relative, their lack of medical knowledge will ordinarily keep them from being ‘then aware that it [was] causing injury to the victim.’ [Citation].” (Downey, supra, 16 Cal.5th at p. 554.) “In such cases, the injury-causing events or conduct are not traumatizing because they are perceived as injury-causing only in retrospect, when the observer is affected by the event’s results.” (Ibid.) “Bird itself made clear that the injury caused by medical negligence is different in this regard from many other types of injury-producing events. We explained that the invisibility of injury-causing events resulting from deficient medical care makes a typical medical negligence case distinguishable from, for instance, a case arising from ‘an explosion, traffic accident, or electrocution.’ [Citation].” (Ibid.)

Here,

Plaintiff alleges the following facts in his complaint: On or about June 6, 2025, the decedent sought

care for right upper extremity weakness from Defendant, and Defendant “failed to appreciate the decedent’s high risk of bleeding despite her ongoing hypoxia, recurrent hemoptysis, and anemia, and proceeded with a right carotid stent placement on June 13, 2025, without first stabilizing her critical condition.” (FAC ¶¶ 2, 6, 8.) Defendant failed to order urgent imaging for the decedent “or evaluate for post-procedural complications after the decedent immediately developed new neurological deficits” and instead “dismissed her symptoms as mere ‘blood pressure variation,’ without investigating the cause of this variation.” (Id. ¶ 8.) Plaintiff also alleges that, “[t]hroughout the evening, defendants, and each of them, failed to recognize the severity of the decedent’s worsening condition until she became critically unstable and required emergent intubation. Only at that point labs were drawn, revealing catastrophic blood loss, and a delayed ultrasound finally identified a massive intraabdominal arterial bleed.” (Ibid.) Plaintiff alleges that the failure to timely recognize, investigate, and treat the decedent’s rapid deterioration resulted in her death on June 14, 2025. (Id. ¶¶ 8–9.)

In support of his NIED cause of action, Plaintiff alleges that he was the decedent’s son and that he was “present at his mother’s bedside for many hours during her hospitalization including specifically on June 13–14, 2025, beginning shortly after her right internal carotid artery stenting procedure and continuing as her condition progressively worsened” and observed his mother’s deteriorating health at that time. (FAC ¶¶ 21–22.) Although Plaintiff sets forth new allegations as to the “unmistakable signs that his mother was dying” without receiving any medical attention and as to his contemporaneous knowledge of the cause of his mother’s death, these allegations are largely conclusory and fail to set forth facts showing that the symptoms and nature of decedent’s deteriorating condition obviously required medical treatment. (FAC ¶¶ 23–25.) Nor do the facts set forth in the FAC show a complete lack of medical attention, as Plaintiff alleges that Defendant’s staff was monitoring her condition throughout this period of time, despite the conclusory allegation that “Plaintiff understood in real time, with every passing minute, that defendants’ refusal to act was directly causing his mother to deteriorate, become paler and less responsive, and fade away and die right in front of him.” (Id. ¶ 24; Downey, supra, 16 Cal.5th at p. 552 [“Bird clearly settled this much: There can be no recovery for emotional distress caused by the negligent

infliction of injury on a third party unless the plaintiff contemporaneously understands that the injury-causing event is in fact causing injury to the victim.”]; Bird, supra, 28 Cal.4th at p. 917 [“Except in the most obvious cases, a misdiagnosis is beyond the awareness of lay bystanders.”].) Accordingly, in liberally construing the FAC, the Court finds that Plaintiff failed to sufficiently allege facts establishing his contemporaneous knowledge that Defendant’s complete failure to provide medical assistance was the cause of his mother’s death. (Compare Ochoa v. Superior Court (1985) 39 Cal.3d 159, 170 with Bird, supra, 28 Cal.4th at pp. 919–920.)

Thus, as Plaintiff failed to show any reasonable possibility of stating a good cause of action for NIED against Defendant, the demurrer to this cause of action is sustained without leave to amend.

B. Fraudulent Concealment and Intentional Misrepresentation

“The elements of fraud are misrepresentation, knowledge of falsity, intent to induce reliance on the misrepresentation, justifiable reliance on the misrepresentation, and resulting damages.” (Reeder v. Specialized Loan Servicing LLC (2020) 52 Cal.App.5th 795, 803 [citing Lazar v. Superior Court (1996) 12 Cal.4th 631].)

“In California, fraud must be pled specifically; general and conclusory allegations do not suffice.” (Lazar, supra, 12 Cal.4th at p. 645.) The particularity requirement necessitates that facts be pleaded that show how, when, where, to whom, and by what means of the representations that were allegedly tendered. (Ibid.) Further, “[a] plaintiff’s burden in asserting a fraud claim against a corporate employer is even greater. In such a case, the plaintiff must ‘allege the names of the persons who made the allegedly fraudulent representations, their authority to speak, to whom they spoke, what they said or wrote, and when it was said or written.’ ” (Id. [quoting Tarmann v. State Farm Mutual Auto. Ins. Co. (1991) 2 Cal.App.4th 153, 157].)

Here, Plaintiff alleges that Dr. Bodhit and Dr. Chang were Defendant’s employees and agent at all relevant times, “acting within the course and scope of their employment and with actual and/or apparent authority to make coroner notifications, sign official death certificates, and author and

sign discharge summaries on behalf of the hospital.” (FAC ¶ 47.) These newly alleged facts appear to cure the prior deficiencies by identifying Dr. Bodhit and Dr. Chang as the individuals who made these statements, their authority to speak on Defendant’s behalf, the recipients and substance of these statements, and when the fraudulent statements were made. (Id. ¶¶ 43, 47.) Regarding Defendant’s challenge as to the sufficiency of the employment or agency allegations as a basis for vicarious liability merited at this stage of the pleadings, as Plaintiff sufficiently pleads the ultimate fact of this relationship in the FAC. (*Ermoian v. Desert Hospital* (2007) 152 Cal.App.4th 475, 502 [“Whether a physician is an agent of a hospital for purposes of vicarious liability is a question of fact.”].) Likewise, Plaintiff’s newly alleged facts regarding the materials omissions made by Dr. Bodhit and Dr. Chang regarding the decedent’s treatment and cause of death are sufficient to support a cause of action for fraudulent concealment. (FAC ¶¶ 32, 37.)

However, Plaintiff’s causation and damages claims lack sufficient factual support. Plaintiff contends that, as a result of the alleged concealment, he believed that the cause of his mother’s death was related to a stroke diagnosis and that no further autopsy was necessary given the coroner’s findings. (FAC ¶ 34.) At the same time, Plaintiff alleges that he wished to have an autopsy because “this was a case clearly needing an autopsy” and that “defendants prevented plaintiff from having an autopsy.” (Id.) The conclusory statement that Defendants prevented him from proceeding with a private autopsy must be explained given that, at the time, Plaintiff believed an autopsy was “clearly” needed. Moreover, given the detailed recitation of the events leading to Plaintiff’s decedent’s death (see, e.g., FAC ¶¶ 8) it is unclear precisely what may have been learned through an autopsy and how that information would have avoided Plaintiff’s claims of “delayed sense of closure, grieving process...delayed justice [and] emotional distress...” (FAC ¶ 38)

Plaintiff’s fraud claims appear to be a creative means of alleging tortious spoliation, a tort that has been uniformly rejected in California. (*Cedars-Sinai Med. Ctr. v. Superior Ct.* (1998) 18 Cal. 4th 1, finding no tort remedy exists for first-party intentional spoliation; *Temple Cmty. Hosp. v. Superior Ct.* (1999) 20 Cal. 4th 464, holding no tort remedy for third-party intentional spoliation; and, *Coprigh v. Superior Ct.* (2000) 80 Cal. App. 4th 1081, concluding no tort remedy exists for negligent spoliation either.) Instead, where a party’s concealment or destruction of evidence causes delay or loss of a claim, the remedy lies in evidentiary sanctions under California Code of Civil Procedure § 2023.030, not

in independent tort damages.

At best, Plaintiff's theory of delayed justice might support a claim of equitable tolling (see, e.g., *Grisham v. Philip Morris U.S.A., Inc.* (2007) 40 Cal. 4th 623, 744), but that remedy is not necessary for this timely-filed suit.

While this Court could not find a California case finding that a delay in filing suit is a compensable damage in a concealment case, even giving Plaintiff the benefit of the doubt and assuming a Code Civ. Proc. § 364 letter was personally served on the day of the death, the earliest a lawsuit could have been filed would have been on September 12, 2025, and this lawsuit was filed just three months later on December 12, 2025. Based on the allegations in the FAC, there is no independent compensable loss for Plaintiff's concealment claims.

Given that Plaintiff has already unsuccessfully attempted to amend these causes of action and given that no suggestion is made how the FAC may be further amended to state a cause of action, the demurrer to the fourth and fifth causes of action is sustained without leave to amend.

RULING

Defendant Dignity Community Care dba Glendale Memorial Hospital and Health Center's Demurrer to Plaintiff's First Amended Complaint is SUSTAINED without leave to amend.